SCML — SITE COMPLICATION MONITORING LOG

SCML · v0.8

Study / Protocol	Site / Code	SCML Record ID / Revision	SCML owner / role
Device / System	Procedure Family / Anatomical Field	Record Opened	Last Updated
Applicable CIP / Safety Procedure / Current Version		Study Category Set / Version (if used)	CSST Review Period / Next Review Date

A Purpose and use

The SCML indexes the site's complications, adverse events, adverse device effects and device deficiencies so they can be compared across cases. Each case is assessed in the Safety and Adverse Effect Reporting Template (SART); the log keeps only the current classification and the links to its sources. Urgent care and safety reporting never wait for this log.

Unit of analysis	Decision supported	Inputs	Outputs
One event, as assessed in its SART case.	How the event is profiled for review across cases, and where its follow-up is sent.	SART case and grade, Intraoperative Checklist and Device Log (ICDL), imaging source.	Indexed events for the Complication Signal Surveillance Template (CSST); status for the PI Oversight Template (POT).

B Recording and source-link guide

Step	What to record here	Source and boundary
Link the event	C: give a unique SCML Event ID; enter the SART case ID and event reference. Use participant and surgeon codes.	Keep the Event ID in C and D. Cite the ICDL and imaging source; do not copy narratives.
Apply a profile	D: select event type, one primary category and timing group. Copy the Clavien-Dindo grade, relationship context and DD finding from the SART.	Use Pending for open items. Other codes come from the study category set.
Review and follow through	E: cite the events or group and the receiving record. F: check the status of an existing action.	Rates and signal decisions sit in the CSST; case follow-up stays in the SART or quality record.

One event keeps one Event ID, even when both the AE and DD pathways apply.

Grade	Clavien-Dindo definition	Example in shoulder arthroplasty
I	Deviation from the normal course needing no treatment beyond simple drugs (analgesics, antiemetics, antipyretics, diuretics, electrolytes) or physiotherapy.	Superficial wound problem treated with dressings; transient nerve palsy resolving without treatment.
II	Needs drug treatment beyond grade I, blood transfusion or parenteral nutrition.	Wound infection treated with antibiotics; thromboembolism treated with anticoagulation.
IIIa	Needs a surgical, endoscopic or radiological intervention not under general anesthesia.	Closed reduction of a dislocation under sedation; aspiration of a hematoma.
IIIb	Needs an intervention under general anesthesia.	Open reduction or revision for instability; liner exchange; washout for deep infection.
IVa	Life-threatening, single organ dysfunction needing intermediate or intensive care.	Pulmonary embolism needing intensive care.
IVb	Life-threatening, multiorgan dysfunction.	Septic shock from a periprosthetic infection.
V	Death of the participant.	

Study / Site Code	SCML Record ID / Revision	Sheet updated (date)	Sheet No. / Total (C)

C Event and Source Index

Enter one row per distinct event or deficiency. **SART case ID / event reference** links the row to its individual source. Use the same clinical term or study term/code as the source. Record the event date and days from the procedure; enter N/A when the interval does not apply. New information about an existing event updates that entry and its source link.

SCML Event ID	SART Case ID / Event Reference	Participant Code	Event Term / Study Code	Event Date / Days from Procedure	Device / Component Code	Surgeon Code	ICDL / Procedure Reference	Imaging Reference

Use the same Event ID in D and mark negative or pending findings explicitly. Keep source details in SART/ICDL; date and initial corrections.

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Structured surveillance profile

Study / Site Code	SCML Record ID / Revision	Sheet updated (date)	Sheet No. / Total (D)

D Structured Surveillance Profile

Use the Event ID from C. Select one event type, primary category and timing group. **Copy the Clavien-Dindo grade, relationship contexts, the DD finding and assessment status from the current SART**; multiple relationship contexts may apply. Record the date/initials checked and use Pending while unresolved.

SCML Event ID	Event Type (from SART)	Primary Surveillance Category	Timing Group	Grade (Clavien-Dindo) (from SART)	Relationship Context (from SART)	Observed DD? (from SART)	SART Assessment Status / Source Check
	☐ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☐ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☐ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☐ Device ☐ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☐ Yes ☐ No ☐ Pending	☐ Pending ☐ Preliminary ☐ Final Date / initials: _____
	☐ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☐ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☐ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☐ Device ☐ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☐ Yes ☐ No ☐ Pending	☐ Pending ☐ Preliminary ☐ Final Date / initials: _____
	☐ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☐ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☐ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☐ Device ☐ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☐ Yes ☐ No ☐ Pending	☐ Pending ☐ Preliminary ☐ Final Date / initials: _____
	☐ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☐ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☐ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☐ Device ☐ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☐ Yes ☐ No ☐ Pending	☐ Pending ☐ Preliminary ☐ Final Date / initials: _____
	☐ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☐ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☐ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☐ Device ☐ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☐ Yes ☐ No ☐ Pending	☐ Pending ☐ Preliminary ☐ Final Date / initials: _____

Relationship contexts reflect the current source assessment. Categories and DD flags are not additional events; follow the CSST counting rule.

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Local review and linked follow-up

Study / Site Code	SCML Record ID / Revision	Sheet updated (date)	Sheet No. / Total (E/F)

E Local Review and Handoff

Log one finding per event or group with a unique Review ID. Select the receiving record and record the decision, linked ID, owner and due date. Use the same Review ID in F. Signal decisions remain in the CSST.

Review ID / Date	Event IDs / Group Reference	Review Finding / Reason for Handoff	Receiving Record(s)	Decision / Linked Record ID / Owner / Due Date
			☐ Safety / SART ☐ CSST ☐ SLCT ☐ SRR ☐ CAPA ☐ POT	
			☐ Safety / SART ☐ CSST ☐ SLCT ☐ SRR ☐ CAPA ☐ POT	
			☐ Safety / SART ☐ CSST ☐ SLCT ☐ SRR ☐ CAPA ☐ POT	
			☐ Safety / SART ☐ CSST ☐ SLCT ☐ SRR ☐ CAPA ☐ POT	

F Linked Action and Handoff Follow-up

Check the existing action in its owning record and cite its outcome and effectiveness conclusion, where applicable. Route a new, overdue or ineffective action through E. Keep a handoff open until the required follow-up is documented.

Review / Event IDs	Existing Action / Handoff Record ID	Current Status	Owner / Due Date	Source Conclusion / Evidence Reference / Checked Date and Initials
		☐ Open / on time ☐ Overdue ☐ Closed ☐ Not assessable		
		☐ Open / on time ☐ Overdue ☐ Closed ☐ Not assessable		
		☐ Open / on time ☐ Overdue ☐ Closed ☐ Not assessable		
		☐ Open / on time ☐ Overdue ☐ Closed ☐ Not assessable		

G Current Index Status and Review Conclusion

Select the highest-priority status: Escalated review active → Control action active → Follow-up or assessment pending → Index current. Cite affected Event IDs and records below. Routine CSST review is not escalation; case closure remains in the SART.

Current Index Status	Select One / Basis	Status Date / Next Review
Index current / no pending item	☐ Entries are reconciled with their sources; no pending assessment, action or handoff is known.	
Follow-up or assessment pending	☐ A source assessment, case follow-up or reconciliation is pending; no higher-priority status applies.	
Control action active	☐ A linked corrective or procedural control action remains open; no escalated review status applies.	
Escalated review active	☐ An urgent or overdue safety handoff, or a triggered signal review, remains open.	

Affected Event IDs / Linked Records / Status Basis:

H Accountable Review and Controlled Record Location

The reviewers confirm that the index was checked against the linked source records and that outstanding handoffs remain identifiable. This review does not replace the investigator's case assessment or any sponsor safety decision. One person may hold more than one site function.

SCML owner / primary reviewer

Name / role:

Signature:
Date:

Clinical / surgical reviewer (if required)

Name / role:

Signature:
Date:

Principal Investigator / accountable approver

Name:

Signature:
Date:

Next review and controlled record location

Review date:

Record location:
Current revision: